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FOCUS GASTROENTEROLOGY

15 April 2026

Dr Tanya Kaouna
Richmond Medical

Dear Dr Kaouna,

Re: Ms Sandra De Oliveira
DOB: 1958-07-16
Address: 470 Bridge Road, RICHMOND VIC 3121

Summary

- **Significant improvement in abdominal pain with the use of Allegron (nortriptyline).**
- **Incidental finding of small pancreatic cysts and gallstones.**
- **For surveillance MRI of the pancreas in 12 months.**
- **Review in 12 months.**
- **No action required by you.**

I had the pleasure of conducting a review with Sandra De Oliveira regarding her abdominal pain. You will be pleased to know that her symptoms have improved significantly. She reports that her pain is now occurring only once a week, which is a substantial reduction compared to her previous baseline.

We discussed her medication regimen, and it is evident that Allegron has been effective in managing her symptoms. Sandra noted a distinct difference in tolerability between brands, reporting significant morning grogginess and a "cloudy" sensation with generic nortriptyline, which is absent when she takes the Allegron brand. I have advised her to request the brand-name version specifically from her pharmacist. We discussed the likely neuropathic nature of her pain, potentially related to the anterior cutaneous nerve, and how nortriptyline facilitates the dampening of these signaling pathways.

Regarding her recent imaging, a CT and subsequent MRI identified small gallstones. However, these are likely incidental as they would not typically respond to nortriptyline. The imaging also

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Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

identified small pancreatic cysts. These were specifically noted to lack any worrisome features or high-risk stigmata. While the radiologist suggested these appearances could be sequelae of prior pancreatitis, Sandra has no clinical history of such an event.

Sandra did mention an issue with blurred vision. While tricyclic antidepressants can theoretically contribute to closed-angle glaucoma or cause sicca symptoms leading to visual changes, this is less common at her current low dose of 10 milligrams. I have nonetheless encouraged her to consult her optometrist for a formal review to ensure there is no significant change in her intraocular pressure or visual acuity.

Impression and Plan

My impression is that Sandra's pain is neuropathic in origin and is currently well-controlled on a low dose of Allegron. I have provided her with a new prescription for Allegron 10 milligrams with instructions to take it earlier in the evening if she experiences any further morning lethargy. Given the pancreatic findings, I have opted for a conservative surveillance strategy. I have provided Sandra with a referral for a follow-up MRI in 12 months to ensure stability. I will review her again in one year's time following that scan, at which point we may consider a trial of weaning her medication.

Kind regards

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Prof Matthew Read